



Infection Prevention and Control Manual

TUBERCULOSIS POLICY

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation-wide	All Staff

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Introduction

Tuberculosis (TB) is a communicable disease caused by the slow-growing bacterium, *Mycobacterium tuberculosis*. Exposure to *M. tuberculosis* is usually via inhalation of aerosols containing viable bacteria. Active TB refers to symptomatic infection associated with tissue invasion. Primary disease is identified as active TB with an onset soon after exposure. Latent TB refers to TB infection where a persistent immune response controls the initial infection, preventing further invasion and signs of active TB. Most patients (approximately 90%) with TB infection will experience latent TB infection (LTBI). Patients with LTBI are not infectious, although may develop active TB disease in the future. Reactivation of latent TB may occur at any time, but is more common within 2-3 years following exposure, with the risk increased by immunosuppression.

The most common focus of primary disease or reactivation is the lungs, and this is known as pulmonary TB. Following initial exposure, *M. tuberculosis* may migrate to other body tissues. Active disease outside the lungs is known as extra-pulmonary TB; this may involve any other part of the body and may occur with or without pulmonary TB. Pleural TB is also classified as extra-pulmonary TB. Extra-pulmonary TB is less infectious than pulmonary TB.

Other species of mycobacteria may cause TB-like illnesses. These are often called “atypical mycobacteria” but are best referred to as non-tuberculous mycobacteria (NTM). They rarely cause significant disease in immunocompetent people and are not readily transmitted from person to person. Like *M. tuberculosis*, NTM bacilli are resistant to decolourisation by acid following laboratory staining and are therefore referred to as acid-fast bacilli (AFB).

NOTE: Tuberculosis is a Notifiable Disease under the Health Act of Western Australia. Notification forms are available [here](#) on HealthPoint.

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Staff Safety	☒
Other:	☐		

Policy Principles

The purpose of this policy is to provide guidance on the infection prevention and management of patients with known or suspected TB, and identification of TB contacts in the healthcare setting. It is designed to ensure the risks to patients, staff and visitors are minimised. Management of TB and management of TB contacts are outside the scope of this policy.

This policy applies to all staff but is aimed at those likely to come in contact with patients known or suspected to have pulmonary and/or extra-pulmonary TB.

1. Early Detection

Early detection of pulmonary TB is essential and will reduce the risk of transmission to patients and staff. The likelihood of pulmonary TB should be assessed on the basis of clinical symptoms/signs and the presence of risk factors for TB. The signs and symptoms that suggest active pulmonary TB include¹:

- Fever and night sweats
- Unintended weight loss
- Feeling generally tired and unwell
- Cough of any duration but especially lasting greater than two weeks
- Haemoptysis.

The main risk factors for TB are²:

- Birth or prior residence in a country with high TB incidence.
- Prior history of TB (latent or active)
- Contact with an active case of TB.
- Being a Healthcare worker
- Being an Aboriginal Australian or born before 1950.

Patients who are immunocompromised (e.g. HIV, malignancy, transplant recipients) are more susceptible to infection with TB as well as progression to active disease. These patients may also present with atypical manifestations of TB, so a higher index of suspicion for TB should be maintained in these patients.

If any patient is suspected of having active pulmonary tuberculosis, they should be placed in a Negative Pressure Isolation Room (NPIR) on airborne precautions immediately. See section 4.1 below for NPIR locations at SCGH.

If active pulmonary TB is suspected please inform the Infection Prevention and Management Unit during office hours.

2. Transmission

M. tuberculosis is most commonly transmitted from person to person via the airborne route. Patients with active pulmonary infection can produce small droplets containing *M. tuberculosis* during coughing, sneezing or other activities such as singing that spray fine particles from the respiratory tract. These particles can remain suspended in the air for several hours, depending on environmental conditions.

The risk of transmission is greatest for pulmonary tuberculosis patients with smear-positive sputum, a cough, those undergoing cough-inducing procedures such as the use of nebulisers, or those with cavitation on chest x-ray. **Smear negative but sputum culture positive pulmonary TB can still transmit TB, albeit at a lower rate.**

The greatest risk to staff and patients occurs from contact with undiagnosed pulmonary TB. Therefore, early detection and a high index of suspicion are essential to minimise the risk of transmission.

Teaching the patient to cover their mouth or nose using a tissue during coughing and sneezing can significantly reduce the risk of transmitting *M. tuberculosis* from patients with pulmonary infection. Additionally, minimising any procedures that induce aerosols also reduces the risk of transmission.

The risk of infection from patients without the risk factors listed above is lower.

Extra-pulmonary TB, without active pulmonary infection, is not readily transmissible. These patients are an infection risk only if they have openly discharging lesions or an indwelling drain with high organism counts, in which case contact precautions are required. Refer to [Section 6. Extra-pulmonary TB](#).

3. Laboratory Diagnosis

Laboratory diagnosis of tuberculosis requires specialised tests that are only performed when specifically requested on the pathology laboratory form. When requested, microscopy for acid-fast bacilli (AFB) will be performed, with results usually available within 24 hours. Culture will also be performed, though the slow growth rate of mycobacteria means that culture-based tests can take several weeks or more to produce a result.

Although no single test is 100% sensitive for tuberculosis, culture remains the most sensitive diagnostic test and is thus the gold standard for the definitive diagnosis of active TB also providing anti-tuberculous drug susceptibilities. Cultures from smear positive specimens usually become positive within one to two weeks whereas smear negative specimens may take up to four weeks. Nucleic acid amplification testing (NAAT) is also available and has a similar sensitivity to AFB microscopy while being specific for *M. tuberculosis* complex.

3.1 Specimens

- Diagnostic specimens should be collected prior to the commencement of treatment
- Three sputum samples (induced or spontaneously produced) should be submitted for AFB microscopy and culture
- Sputum specimens are best collected on separate days, preferably early in the morning, prior to eating and drinking; however, patients who are very productive of sputum can have three specimens collected at least 8 hours apart within 24 hours
- Sputum collection should be considered in all suspected cases of active TB, even in suspected extra-pulmonary cases
- For patients who are not productive of sputum and require induced sputum collection, a minimum of two induced sputum specimens are required
- Bronchoscopy or broncho-alveolar lavage may be performed to obtain lower respiratory tract specimens for AFB microscopy and culture; if bronchoscopy is performed, at least one additional lower respiratory tract specimen should be collected at a separate time.

If the sputum or bronchoscopy specimens under suspicion for tuberculosis have AFBs present on microscopic examination, the patient is considered 'smear positive'. When AFB microscopy and NAAT are both positive, the diagnosis of tuberculosis can be considered to be established. If all three sputum specimens are negative for AFBs, the patient is considered 'smear negative'². If the suspicion for pulmonary TB remains high, further testing (e.g. NAAT) can be considered.

Induced sputum collection for tuberculosis must be performed in a NPIR. If the patient requires a bronchoscopy, all staff present in the room during the bronchoscopy procedure must wear a fit tested and fit checked Particulate Filter Respirator (PFR), protective eyewear, long sleeved fluid repellent gown and gloves. Refer to Section 4.14.

NOTE: TB is in the differential diagnosis for some patients admitted by the Respiratory Physicians for bronchoscopy procedures in the Short Stay Unit (SSU). The Respiratory Physician will assess the likelihood of pulmonary TB for each patient prior to admission and notify ward G54 and the Infection Prevention and Management Unit if admission to the

NPIR is required. If Airborne Precautions are not required, the patient should be managed following Standard Precautions in the SSU.

4. Management of patients with suspected or confirmed pulmonary tuberculosis

Airborne (blue card) and standard precautions are required for patients with untreated suspected or confirmed pulmonary TB with any of the following:

- Smear positive sputum or sputum results pending
- Cough or haemoptysis.
- Chest imaging suggestive of pulmonary TB (e.g. cavitation, upper lobe infiltrates)

Refer to Appendix 1. [Management Guideline for Suspected Tuberculosis.](#)

For patients with extrapulmonary TB, refer to [Section 6. Extrapulmonary Tuberculosis.](#)

4.1 Airborne precautions

Activated Negative Pressure Isolation Room (NPIR)

Patients admitted to the NPIR for suspected pulmonary TB must have a Respiratory Department or Infectious Diseases Service consultation within 24 hours to determine ongoing need for airborne precautions.

Airborne precautions are to be continued after treatment is commenced until the patient is no longer considered infectious, see [Section 5. Patients with sputum culture in progress.](#)

Negative Pressure Isolation Rooms (NPIR) are located on:

- Ward G54 x 2 – rooms 9 and 10.
- Ward G71 x 1 – room 15 and room 14 used for the anteroom.
- C14 x 4 – rooms 2, 7, 25 and 26.
- Intensive Care Unit x 2 – N9 and N10.
- Emergency Department x 2.

In addition to standard precautions, airborne precautions include:

- Placing a blue 'Airborne precaution' sign outside the NPIR - explain reason for isolation to patient and family.
- Wearing Airborne PPE including a fit tested and fit checked Particular Filter Respirator, protective eyewear, long sleeve gown and gloves when entering the room. Don and doff PPE in the anteroom.

NOTE: There is no NPIR at Osborne Park Hospital. Any patient suspected or confirmed as having pulmonary TB while at OPH, must be considered for transfer to SCGH. Refer to section 4.13.

Advice to patients

Direct contact with other patients should be avoided. A surgical face mask must be worn by the patient whilst outside the NPIR. The patient may be allowed outside in the hospital grounds (not the 5th Floor courtyard) but should avoid going to the hospital shops or restaurant.

Patients should be advised to cover their mouth and nose with a tissue when coughing or sneezing and perform hand hygiene.

4.2 Hand Hygiene

Perform hand hygiene in accordance with the '5 moments for hand hygiene'. Use handwash solution or alcohol-based hand rub.

4.3 Personal Protective Equipment

For additional information on PPE refer to the SCGOPHCG Standard and transmission-based precautions Policy

Correct donning and doffing procedures, including hand hygiene must be performed in the anteroom to the NPIR.

4.4 Patient care equipment

- Keep equipment in room to a minimum.
- Disposable, single use equipment e.g. tourniquet and sphygmomanometer cuff should be used where possible and disposed of into appropriate waste stream after use
- Dedicate non-critical items, e.g. stethoscope to the room
- Reusable medical devices/equipment is to be cleaned with a combined detergent/disinfectant wipe e.g. Clinell® wipe before use on another patient
- Unused disposable items e.g. dressings, kidney dishes to be discarded on patient discharge.

Note: Spirometers should not be used on smear-positive patients unless requested by the Respiratory Physician.

4.5 Environmental Cleaning

Environmental cleaning and disinfection are essential to prevent the transmission of infection.

Staff performing daily cleaning must wear Airborne PPE, including a fit tested and fit checked PFR, protective eyewear, long sleeve gown and gloves when entering the room.

A daily disinfection clean with a combined detergent and disinfectant solution e.g. Chloradet® is required.

Terminal cleaning of room

On patient discharge maintain negative pressure in NPIR for at least 30 minutes (or as otherwise indicated in [Table 1. Length of time room to be left prior to next patient.](#)

Cleaning staff wearing airborne PPE can enter the room once the room has been vacated by the confirmed or suspected TB patient.

- All disposable items in the room are to be discarded on discharge
- A hydrogen peroxide disinfection clean (Nocospray®) is not required on discharge.

Cleaning of other areas e.g. Radiology

- For rooms with short term patients e.g. CT scan, Xray's, MRI, clean and disinfect all touched surfaces within 1.5 metre zone of the patient between cases (use combined detergent and disinfectant wipes or use Chloradet. If patient cannot tolerate wearing a mask refer to Table 1 for length of time room to be left prior to next patient. Cleaning can commence after exit of the patient.

Table 1. Length of time room to be left prior to next patient.

Area	Length of time room to be left prior to next patient
Single room	60 minutes (30 minutes if HEPA filter in use)
Negative pressure isolation room	30 minutes
Procedure rooms: Interventional radiology Rooms 18, 19,20 Cardiac catheter Laboratories 1, 2, 3	20 minutes
NIISWA Procedure Rooms 21, 22	60 minutes
Operating theatres	20 minutes
Bronchoscopy Room, G75	20 minutes
All other areas	60 minutes (30 minutes if HEPA filter is used)
After an AGP	30 minutes (if NPIR), 60 minutes if standard room

4.6 Linen management

- Handle soiled linen with minimum agitation to avoid contamination of the air, surfaces or persons
- Soiled linen must be placed in the linen carrier at the point of use
- Linen that is wet or soiled with blood or body substances must be placed inside a plastic bag before being placed in the linen carrier
- Ensure all linen bags are securely tied prior to transporting from the patient room to collection area
- Stockpiling supplies of linen in the patient room is not to occur and any unused linen must be sent for laundering and not returned to general use.
- Perform hand hygiene following the handling of used linen.

4.7 Body substance spills

Follow standard precautions.

4.8 Disposal of waste

Follow the SCGOPHCG Waste Management Policy.

- Use standard precautions when handling waste
- Any waste that is contaminated with blood and/or body fluids is classified as clinical waste
- Most of the waste including PPE can be classified as general waste
- All waste shall be bagged and securely sealed prior to exiting from the patient room.

4.9 Catering

- At a minimum, catering staff must wear a fit tested and fit checked PFR and eye-protection prior to entering the room.
- Standard precautions should be used when handling used crockery and cutlery returned to the dishwashing area
- Disposable crockery and cutlery are not required

- The combination of hot water and detergents used in automatic dishwashers is sufficient to decontaminate these items
- Unopened food items or food waste is to be discarded into general waste.

4.10 Transporting Patients/Transfer to other Departments/HCFs

- Transfer of patients must be kept to a minimum
- The receiving HCF/department must be notified prior to transfer that the patient is confirmed or suspected of having TB and on airborne precautions. Confirm when they are ready to receive the patient
- Patient to wear a surgical mask, where tolerated for the duration of the transfer.
- Patients requiring oxygen therapy should be transitioned to nasal prongs under a surgical mask if possible. If the patient is unable to transition to nasal prongs a surgical mask should be placed over the Hudson mask prior to transport
- Encourage patient to follow respiratory hygiene and cough etiquette
- All members of the transfer team must don fit tested and fit checked PFR
- Patient must be taken straight to and returned from clinical departments and must not wait in any communal areas. If possible, place the patient at the end of the list
- **Ensure no other staff or visitors are in the lift during transport**
- A disinfection clean of the area and equipment used on the patient is required once the patient has left the relevant department e.g. Radiology. Refer to Section 4.5.

4.11 Transport of Laboratory Specimens

Follow standard precautions.

4.12 Visitors

Close contact (family members/household contacts) immediately prior to admission:

- These contacts will be assessed by staff from the Anita Clayton Centre (WA Tuberculosis Control Program) and follow up organised.
- If visiting they must be instructed on hand hygiene and the use of non-fit tested PFR (3M Aura 1870+ recommended).
- It is recommended children under 15 years of age do not visit as they are at higher risk of progression of the disease.

No previous close contact with the patient immediately prior to admission:

- Ask to postpone visit until the patient is no longer an infection risk.

4.13 Management of the deceased

Follow Nursing Practice Guideline Last Offices.

- Staff to wear PPE – gown, fit tested and fit checked PFR, protective eyewear and gloves when preparing the body for transport
- Avoid unnecessary manipulation of the body that may expel air
- A surgical face mask is to be placed on the deceased prior to movement of the body and for the duration of care until the body is placed in a leak proof body bag
- Inform the mortuary (SCGH) or Funeral Director (OPH) prior to transfer of the deceased.

NOTE: If patient already considered non-infectious (released from Airborne precautions), Standard precautions apply.

4.14 Management of patients undergoing bronchoscopy

- All staff present in the Bronchoscopy Suite or Operating Room must wear a fit tested and fit checked PFR, protective eyewear, gown, and gloves.
- Doors must be closed, and no staff are to enter or exit during the procedure.
- Patient to be recovered post-procedure in the Bronchoscopy Suite or Operating Room then transferred to a NPIR until discharged.
- Cleaning staff wearing PPE (gown, fit tested and fit checked PFR, protective eyewear and gloves) can enter room once the room has been vacated by the confirmed or suspected TB patient.

If a NPIR is not available post-bronchoscopy, patients with suspected TB requiring a bronchoscopy may be risk assessed by Respiratory Medicine for possible post-procedure management on G54. This would include patients identified as being a lower risk of infectiousness who are sputum smear negative pre-bronchoscopy, have no evidence of cavitation on chest X-ray, can tolerate wearing a surgical mask and are otherwise well and so are expected to be discharged within 4 hours of admission.

Patients who are identified as being suitable for post-bronchoscopy recovery on G54, are to be managed in rooms 5 and 6 only with the following precautions in place:

- Place airborne precautions (blue sign) at the door entry.
- Door to room to remain closed.
- Staff entering room to wear PPE for airborne precautions, including a minimum of fit tested and fit checked PFR.
- Patient to wear a surgical mask if possible.
- Portable air purifier to be placed in the room.
- Room to remain vacant for 30 minutes after patient discharged with portable air purifier running.

4.15 Management of patients in the Operating Theatre/Procedural Area

- In addition to surgical PPE all staff in the room must wear a fit tested and fit checked PFR and protective eyewear (if not part of the surgical team, gown and gloves are also required)
- Intubation/extubation is to be undertaken in the Operating or Procedure room (cardiac catheter lab, Radiology/NIISWA)
- Doors must be closed, and no staff are to enter or exit during the procedure
- Patient to be recovered in Operating or Procedure Room
- Cleaning staff wearing PPE (gown, fit tested and fit checked PFR, protective eyewear and gloves) can enter room once the room has been vacated by the confirmed or suspected TB patient.

Refer to Table 1 for length of time room to be left prior to the next patient.

4.16 Management of patients undergoing other aerosol-generating procedures

- Aerosol generating procedures must not be undertaken on patients with confirmed or suspected TB unless strictly necessary and authorised by the Medical Consultant
- If AGPs are required e.g. induced sputum collection for TB they must be performed in a NPIR
- All staff must wear PPE (gown, fit tested and fit checked PFR, protective eye wear and gloves) during the procedure.

4.17 Duration of airborne precautions

Pulmonary TB patients remain on airborne precautions for two weeks after commencing anti-tuberculosis therapy. Airborne precautions may cease after two weeks if there has been an adequate clinical response to treatment, as determined by a Respiratory or Infectious Diseases Physician, or the Infection Prevention and Control Unit in consultation with the treating medical team. Failure to improve clinically on treatment may be due to multi-drug resistant tuberculosis.

4.18 Patients with suspected or confirmed pulmonary TB at OPH

- Inform on-call Microbiologist via switch and/or the Infection Prevention and Control nurse to determine if transfer to SCGH and/or collection of sputum specimens is required
- If awaiting transfer, patient to wear a surgical mask. If there is a delay in transfer, move patient to a single room and keep the door closed
- Staff caring for patient to wear PPE (gown, fit tested and fit checked Particulate Filter respirator (PFR), protective eyewear, gloves). Refer to Section 4.14
- Ambulance services should be informed of patient's diagnosis and the patient should wear a surgical mask during transfer
- Investigations and treatment that may induce coughing must be deferred where possible, until active TB has been excluded or the patient is in a NPIR at another HCF
- After discharge, follow Environmental cleaning – terminal cleaning guidelines in Section 4.5.

5. Patients with sputum culture in progress

Removing untreated pulmonary TB cases from airborne precautions puts staff and other patients at risk. Smear negative pulmonary TB cases are still infectious, and if *M. tuberculosis* is subsequently cultured from AFB negative sputum where the patient was not managed on airborne precautions throughout their hospital admission, contact tracing is required.² Therefore, patients initially identified as having suspected pulmonary TB who have not received two weeks of TB treatment and where sputum culture is still in progress should only be removed from airborne precautions following assessment by a Respiratory Medicine or Infectious Diseases Consultant, or in discussion with the Infection Prevention and Management Unit or on-call Clinical Microbiologist.

It is sometimes difficult to fully exclude tuberculosis in smear negative patients where it remains part of the differential diagnosis. In cases where it is felt to be very unlikely, especially where continued airborne isolation may compromise medical care, it may be appropriate for smear negative patients to be managed on droplet precautions (green card) or in a single room on standard precautions. This decision is only to be made by the Respiratory Medicine or Infectious Diseases Consultant, in discussion with the Infection Prevention and Control Unit or on-call Clinical Microbiologist.

Refer also to Section 4.14 Management of Patients undergoing Bronchoscopy.

6. Extra-pulmonary Tuberculosis

Additional isolation precautions are not usually required for these patients unless the patient has a discharging lesion or an indwelling drain in a cavity suspected or confirmed to contain *M. tuberculosis*, in which case standard and contact precautions must be followed. All patients with suspected or confirmed extra-pulmonary TB should have chest imaging performed (e.g. chest X-ray) and should be assessed for the possibility of co-existent pulmonary TB. This assessment will normally be performed by a Respiratory or

Infectious Diseases Physician. If the assessment determines the patient may have pulmonary TB, they should be managed as for suspected or confirmed pulmonary TB.

Invasive procedures performed on or near sites of extra-pulmonary TB infection may increase the risk of aerosol transmission. All patients with extra-pulmonary TB undergoing surgery or invasive procedures should be discussed with the Infection Prevention and Management unit for a risk assessment and consideration for airborne precautions.

7. Latent TB

Latent TB refers to TB infection where a persistent immune response controls the initial infection, preventing further invasion and signs of active TB. Most patients (approximately 90%) with TB infection will experience latent TB infection (LTBI). Patients with LTBI are not infectious and can be managed with standard precautions.

8. Non-Tuberculous Mycobacterial Infections

No additional isolation precautions are required for these patients.

9. Multi-drug resistant Pulmonary Tuberculosis (Resistant to Rifampicin and Isoniazid)

Tuberculosis resistant to anti-tuberculosis agents is more difficult to treat and results in a longer period of potential infection risk to staff and patients. There are no completely reliable indicators for risk of multi-drug resistant mycobacteria, but patients who have had incomplete therapy (e.g. two agent regimens in developing countries, or poor compliance with locally administered therapy) are at higher risk. Rifampicin resistance may be detected by molecular testing but confirmation by antibiotic susceptibility testing usually takes several weeks to complete in the laboratory. Multi-drug resistant cases can also be suspected by a failure to respond to therapy. Hospital staff must directly observe the administration of anti-tuberculosis therapy to these patients.

10. Follow-up of Contacts

When a hospital inpatient or staff member is diagnosed with TB, follow-up of other staff and patients who have had 'significant contact' with the index case is necessary².

'Significant contact' is defined by either of the following:

- Sharing the same air space as the index case for a cumulative time of at least 8 hours without airborne precautions (e.g. a routine shift of 8 hours on a ward with an infectious TB patient who was not in NPIR)
- Involvement in a procedure that confers increased risk (e.g. sputum induction, bronchoscopy, intubation) where airborne precautions had not been implemented

Patients who have shared a room with an index case for at least 8 cumulative hours are considered a 'close contact'. Other patients or staff who have been in the same air space but for less than 8 cumulative hours and have not been present during an aerosol generating procedure are usually considered a 'casual contact'. Follow-up is not generally required for 'casual contacts' of smear negative pulmonary TB cases. However, determination of 'significant' versus 'casual contact' and the required follow-up will be made on a case-by-case basis.

9.1 Management of inpatients who have significant contact with a pulmonary tuberculosis case

Follow-up of inpatient contacts will be undertaken by the Infection Prevention and Control Unit in consultation with the WA TB Control Program.

9.2 Management of staff who have significant contact with a pulmonary tuberculosis case

Follow-up of staff contacts with either a patient or another staff member with pulmonary tuberculosis who has not been isolated, or where a breach in infection control procedures has occurred will be undertaken by the Health Safety and Wellness Department in consultation with the Anita Clayton Centre.

11. Staff Health

10.1 Pre-employment assessment

All employees involved in clinical care will be assessed for risk of TB prior to commencing clinical work by Occupational Safety and Health. Refer to SCGOPHCG Infection Prevention and Control Policy: Healthcare Worker Health and Immunisation Policy.

10.2 Pregnant Staff

Pregnant staff must not care for patients with suspected or confirmed pulmonary TB.

10.3 Immunosuppressed staff

Immunosuppressed staff must not care for patients with suspected or confirmed pulmonary TB.

10.4 BCG Vaccination

BCG vaccination is not routinely recommended for healthcare workers in Western Australia.

Compliance, monitoring, and evaluation

The Infection Prevention and Management nurses undertake daily monitoring and surveillance to identify potential TB cases and ensure appropriate precautions are implemented.

Related documents

- Health Services Act 2016
- Public Health Act 2016
- [SCGOPHCG IPM Manual: Standard and transmission-based precautions policy](#)
- SCGOPHCG IPM Manual: Healthcare Worker Health and Immunisation Policy
- SCGH Respiratory Protection Program

References

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Authorisation & Version Control

Policy Sponsor	Nurse Co-Director Medical Division				
Policy Contact	Coordinator of Nursing, Infection Prevention and Control Unit				
Date First Issued:	01/08/1986	Last Reviewed:	October 2022	Review Date:	October 2025
Version No.	5.2 Revised February 2025				
Authorised by:	Hospital Infection Prevention and Control Committee			Date:	17/11/2022
Standards Applicable:					

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form.

Appendix 1: Management Guideline for Suspected Tuberculosis

